

Sleep regimen for tonight

Action handout based on OE #26 (5/23) Tier 1 ranking · 2026-05-23

For Dad, Mom, Leah, David,
Debbi, family team

RECOMMENDED FOR TONIGHT (TIER 1, OE 5/23)

Lemborexant 10 mg + melatonin 5 mg HS + lorazepam 0.5 mg PRN rescue at bedside

Specifics that matter:

- **Lemborexant 10 mg as a SINGLE dose at bedtime** — not split into two doses like last night. On an empty stomach (no food within 3 hours). Taken when planning ≥ 7 hours of bed time remaining before waking.
- **Melatonin reduced from 10 mg to 5 mg** — evidence for benefit above 5 mg in elderly is absent; 10 mg is supraphysiologic with no demonstrated additional benefit.
- **Lorazepam 0.5 mg at bedside as PRN rescue ONLY** — not given at bedtime alongside lemborexant. Take only if not asleep within ~ 90 minutes. One dose tonight is not habit-forming; the family-concern about dependence trajectory applies to scheduled/daily use, not a single rescue.
- **Fall precautions:** nightlight on, clear path to bathroom, ask Mom for help if getting up at night.

Why this regimen, not the others

WHY LEMBOREXANT 10 MG SINGLE DOSE (NOT SPLIT)

Last night's lemborexant 10 mg total taken as half-tablet (5 mg) + later half-tablet (5 mg) was pharmacokinetically suboptimal. Lemborexant has a half-life of ~ 18 hours and is designed to be taken as a single dose immediately before bedtime; the FDA label specifies single-dose administration. The first 5 mg partially cleared before the second 5 mg achieved peak; neither dose alone reached the receptor occupancy of a single 10 mg bolus. **Lemborexant has not been adequately trialed yet** — last night's failure shouldn't be read as the drug failing for Dad. Lemborexant carries the lowest fall risk (no postural-stability impairment in elderly studies) and the cleanest pre-ECT profile of the options.

WHY MELATONIN 5 MG RATHER THAN 10 MG

Evidence for melatonin benefit above 5 mg in elderly is absent; doses above 5 mg are supraphysiologic with no demonstrated additional sleep benefit. Maintaining the 10 mg dose adds no advantage and may produce daytime grogginess.

WHY LORAZEPAM AT BEDSIDE AS RESCUE, NOT AT START OF NIGHT

Combining lemborexant + lorazepam at the start of the night risks (a) muddying the assessment of whether lemborexant works for Dad; (b) additive sedation/fall risk; (c) reinforcing a sleep-requires-Ativan pattern. Holding 0.5 mg in reserve as a rescue preserves the option without committing to the combination. One dose if truly needed at 90 minutes of non-sleep is appropriate; a second dose mid-night is not.

What to avoid tonight (and why)

DO NOT — TIER 4 / CONTRAINDICATED OPTIONS FOR TONIGHT

- **DO NOT use Seroquel (quetiapine) tonight at any dose without a fresh baseline ECG first.** Dad has ≥ 4 QT risk factors (age, bradycardia HR 49–53, LAFB + borderline 1° AV block + QRS 120, polypharmacy with QT-active agents). The FDA label explicitly warns against quetiapine use with bradycardia and conduction abnormalities. Last ECG is over a year old. Hui 2025 found low-dose quetiapine in elderly carried HR 3.1 mortality, HR 8.1 dementia, HR 2.8 falls vs trazodone.
- **If Dr. L proposes 50 mg quetiapine, that is meaningfully worse than 25 mg.** Elderly clearance is 30–50% reduced, so 50 mg produces effective levels equivalent to ~65–100 mg in a younger adult. Lancet Neurology 2020/2025 LBD reviews recommend lowest effective dose. If quetiapine ends up being used, start at 12.5–25 mg, not 50 mg — and only after fresh ECG.
- **DO NOT re-introduce doxepin tonight.** Nordoxepin (active metabolite, 31-hour half-life) is still meaningfully on board through ~5/26 from the recent ~17-night course. Re-introducing 6 mg would add to the residual, potentially producing supra-therapeutic exposure. Also: FDA geriatric max is 3 mg, not 6 mg. If doxepin is desired later, wait until ~5/28 and start at 3 mg.
- **DO NOT escalate to olanzapine if lemborexant fails** — given the 4/2026 near-panic-attack rebound experience and pending Lewy-spectrum workup. Olanzapine carries substantially higher parkinsonism risk than quetiapine and is categorically avoided in confirmed DLB per Lancet Neurology 2025.
- **DO NOT combine lemborexant + lorazepam at the start of the night** — lorazepam is rescue-only, not co-administered.

Fallback for tomorrow night if tonight fails

TIER 1-ALT FOR NIGHT 2 (OE #26)

Trazodone 50 mg + lorazepam 0.5 mg + melatonin 5 mg HS. This is the OE-supported fallback if lemborexant 10 mg single-dose fails to produce sleep tonight. The trazodone provides histaminergic + 5-HT_{2A} sedation; the scheduled lorazepam 0.5 mg adds GABAergic floor for 2–4 nights pre-ECT (negligible dependence risk at that dose/duration; 0.5 mg = 5 mg diazepam equivalent, well below the 20 mg threshold associated with ECT-seizure-compromise per Pariwatcharakul 2025). The lorazepam would be discontinued before ECT initiation 1–2 weeks out.

Monitoring tonight

- Note time lemborexant taken; time of sleep onset; any awakenings; total sleep time.
- If lorazepam rescue used: note time, dose, and effect.
- Watch for excessive sedation, confusion, or unsteadiness — especially if lorazepam is used.
- Watch for the shaking pattern overnight or AM — does it change with the regimen?
- Note any new dream content or RBD-like episodes (Dayvigo increases REM sleep; theoretical concern for RBD-spectrum patients).

Failure criteria for switching on night 2

- If sleep tonight is <3 hours total: switch to Tier 1-alt (trazodone + lorazepam + melatonin) for night 2.
- If sleep tonight is 3–5 hours: retry the same Tier 1 regimen for night 2 — one night isn't a fair evaluation.
- If sleep tonight is ≥6 hours: hold the regimen; this is working.

If Mom needs to talk to Dad about tonight's plan

FOR MOM TO DAD

"Tonight we're going to take the Dayvigo as a single tablet at bedtime — not split like last night. Last night's split-dose didn't give the medicine a fair chance to work. We have Ativan as a backup if you're not asleep by 90 minutes; one dose tonight is fine and isn't a habit problem. One night doesn't tell us much — if tonight isn't great, we have a clear plan for tomorrow. Mom can come hold your legs if the shaking gets bad like this morning."

This handout reflects the OE #26 (5/23) Tier 1 ranking based on FDA labels, the SUNRISE-1/SUNRISE-2 trials (lemborexant in elderly), the Pariwatcharakul 2025 dose-dependent benzodiazepine-ECT analysis, the Hui 2025 quetiapine-in-elderly safety analysis, the Wang 2024 QT-prolongation data, the Højlund 2022 MACE analysis, Lancet Neurology 2020/2025 LBD landscape reviews, the doxepin / nordoxepin FDA label, and the De Crescenzo 2022 Lancet network meta-analysis. The recommendations are interpreted for Dad's specific profile (age 76, severe TRD, ≥4 cardiac QT risk factors, findings under evaluation for possible prodromal Lewy-spectrum disease, residual nordoxepin from recent doxepin course, activation-intolerance pattern, planned U-M ECT 1–3 weeks out).